

SAMPLE EDITION · SEPTEMBER 2026

US Healthcare Market Report 2026

FDA, CMS, PBMs and commercial access intelligence for the world's largest healthcare market
— pharmaceuticals, medical devices, IRA price negotiation and hospital purchasing.

COVERAGE

United States

FOCUS

Pharma · MedTech · Access

USE

Launch & payer planning

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This PDF is a **gated sample** of BioNexus US healthcare intelligence, aligned to figures published on bionixus.com/usa-healthcare-market-report (May 2026) and the on-page market-intelligence KPIs. It is a board briefing, not a syndicated SKU feed and not legal, medical or investment advice.

WHAT THIS SAMPLE IS — AND IS NOT

It is drawn from BioNexus published market intelligence: sizing bands, regulators, access steps, epidemiology and named hospital systems. Ranges are published BioNexus bands, not a single-point vendor forecast. Where the page FAQ and the KPI panel differ, both are shown and the FAQ band is used as the download-attached headline figure.

Live page: bionixus.com/usa-healthcare-market-report · Related: [/canada-healthcare-market-report](#) · [/brazil-healthcare-market-report](#) · [/gcc-pharma-market-report-2026](#)

02

United States at a glance

336M POPULATION 2026	~\$4.5T HEALTHCARE MARKET 2026	~\$615B PHARMA MARKET 2026	~\$180B DEVICES MARKET 2026
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The United States is the world’s largest healthcare market and accounts for approximately **45% of global pharmaceutical revenues**. Public payers (Medicare 65+, Medicaid, CMS) sit alongside employer-sponsored insurance, ACA marketplace plans and commercial managed care. CMS administers approximately **USD 1.8 trillion** in annual expenditure. About **92%** of the population has some form of health insurance.

PUBLISHED SERIES	HEALTHCARE	PHARMA	DEVICES
Page hero / FAQ (this sample's headline)	USD 4.4–4.6T · ~\$4.5T · 17.6% of GDP	USD 590–640B · ~\$615B	~\$180B
On-page market-intelligence KPIs	USD 5.0–5.2T · 17.8% of GDP · USD 14,900 / capita	USD 615–650B	USD 175–200B

SYSTEM ARCHITECTURE

Population 336 million (US Census Bureau). GDP per capita USD 82,000 (IMF 2025). ~6,120 hospitals (non-profit ~2,900; for-profit ~1,200; government ~1,000 — AHA 2024). ~920,000 beds (2.7 per 1,000). Medicare ~65M; Medicaid ~90M; remainder commercial.

REGULATORS AND PAYERS

FDA CDER (drugs), CBER (biologics), CDRH (devices). CMS for Medicare/Medicaid. PBMs — UnitedHealth/Optum, CVS/Caremark, Express Scripts/Cigna. GPOs — Vizient, Premier, HealthTrust (~80% of hospital purchasing).

Hero chips on /usa-healthcare-market-report. KPI panel: BioNexus market intelligence (USA). FDA/CMS/PBM detail from the same page FAQ.

03 US pharmaceutical market

BioNexus sizes the US pharmaceutical market at **USD 590–640 billion in 2026** on the live report FAQ (hero midpoint **~\$615B**). The KPI panel uses **USD 615–650 billion**. Either way this is the world's largest national market by value.

AUTHORISATION IS NOT ACCESS

FDA approval grants market authorisation only. Commercial access still requires PBM formulary placement, CMS coverage for Medicare, GPO contracts for hospital products, and — for high-spend Medicare drugs — IRA Maximum Fair Prices.

FDA PATHWAYS

NDA (21 CFR 314) and BLA (21 CFR 601), eCTD. PDUFA: 10 months standard, 6 months priority review. Breakthrough Therapy allows rolling review. Advisory Committee meetings are non-binding but highly influential.

LARGEST THERAPY AREAS BY COMMERCIAL SPEND (PUBLISHED FAQ)

Oncology (largest and fastest-growing — immuno-oncology, targeted therapy, CAR-T). Immunology and biologics (TNF inhibitors at scale; IL-17/23 and JAK inhibitors growing; biosimilars accelerating since 2023). GLP-1 / obesity and diabetes (semaglutide and tirzepatide; FAQ notes a potential **USD 50B+** category by 2030). Cardiovascular (novel heart-failure agents, PCSK9, novel anticoagulants). Rare disease and gene therapy (orphan designations; gene therapies at USD 1M+ list prices).

The Inflation Reduction Act is the most significant structural change to US pharmaceutical commercial models since the Medicare Modernization Act of 2003 — reshaping launch strategy, pricing architecture and international reference-pricing dynamics.

bionixus.com/usa-healthcare-market-report — FAQ and executive summary (published 27 May 2026).

04 US medical devices market

~\$180B HERO CHIP 2026	\$175–200B KPI PANEL (MDMA/ADVAMED)	FDA CDRH DEVICE REGULATOR
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The live report hero uses **~\$180B** for US medical devices in 2026. The on-page KPI cites **USD 175–200 billion (MDMA/AdvaMed)**. This sample keeps both. Hospital access is GPO-mediated: Vizient, Premier and HealthTrust control about **80%** of US hospital purchasing (3–9 months to contract).

DEVICE COVERAGE AFTER FDA CLEARANCE

- 1 **FDA CDRH clearance / approval** — risk class and 510(k), De Novo or PMA pathway.
- 2 **CMS coverage** — National Coverage Determination (~6 months) or automatic Part B/D coverage. TCET (Transitional Coverage for Emerging Technology) for qualifying devices.
- 3 **GPO contract** — Vizient, Premier, HealthTrust. Critical for hospital drugs and devices.
- 4 **IDN / hospital value-analysis committee** — parallel to GPO; determines on-shelf conversion.

Capital-equipment, consumables and IVD all sit in this commercial stack. ICER assessments can influence payer decisions but have **no statutory authority**.

Device KPI: BioNixus USA market intelligence (MDMA/AdvaMed). Access steps: same KPI registration path. Hero chip: /usa-healthcare-market-report.

05 Therapy landscape

Therapy (KPI Panel)	2026 Size	Growth	What moves the market
Oncology	USD 145–165B	11% CAGR	FDA Oncology Center of Excellence; 660+ oncology approvals 2017–2023; PD-1/PD-L1 USD 35B+
Immunology & biologics	USD 90–100B	12% CAGR	Adalimumab biosimilar launch 2023; IL-17/23; JAK inhibitors
Cardiovascular	USD 75–85B	7% CAGR	TAVI, PCSK9, SGLT-2 HFrEF/CKD indications
Diabetes & metabolic	USD 60–70B	15% CAGR	Tirzepatide / semaglutide; IRA insulin cap USD 35/month Medicare
Rare disease	USD 35–40B	14% CAGR	7,000+ rare diseases; FDA Rare Disease Innovation Hub; gene therapy

EPIDEMIOLOGY (CITED ON-PAGE)

Condition	Published Stat	Source on page
Cancer	~2.0 million new diagnoses/year; breast, lung, colorectal, prostate most prevalent	ACS Cancer Facts & Figures 2024
Cardiovascular disease	~800,000 myocardial infarctions/year; #1 cause of death	AHA Heart Disease and Stroke Statistics 2024
Obesity	41.9% of adults obese — primary GLP-1/GIP demand driver	CDC NHANES 2023

Therapy sizes and epidemiology: BioNixus USA market-intelligence panel on /usa-healthcare-market-report. FAQ therapy ranking is qualitative and matches this order at the top of the spend stack.

06 FDA, CMS and PBM sequence

10–24 mo REGULATORY APPROVAL	6–18 mo PAYER LISTING	19–51 mo LAUNCH TO ACCESS
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Breakthrough / priority review can compress the total clock to about **12–18 months**. There is no single national HTA body — payer-by-payer formulary negotiation is the primary access mechanism.

- 1 **Pre-NDA/BLA Type B meeting** — FDA; 90 days post-request. Align data; rolling review if breakthrough.
- 2 **NDA/BLA submission** — CDER or CBER. Day 0. eCTD.
- 3 **FDA technical review** — PDUFA 10 months standard / 6 months priority.
- 4 **Advisory Committee (if warranted)** — ~8–9 months into review. Public PI discussion.
- 5 **FDA approval letter** — PDUFA target action date.
- 6 **CMS coverage** — NCD ~6 months, or automatic Part B/D. Medicare Part B physician-administered infusions use ASP + 6%.
- 7 **Commercial PBM formulary** — Caremark, Express Scripts, OptumRx. 6–18 months. Step therapy and prior authorisation are common.
- 8 **GPO contract** — Vizient, Premier, HealthTrust. 3–9 months.

Registration steps and accessTimeline: BioNexus USA market intelligence on the live report page.

07 IRA, PBMs and formulary access

INFLATION REDUCTION ACT (2022) — THREE PUBLISHED CHANGES

- 1 **Medicare drug price negotiation** — CMS negotiates Maximum Fair Prices for the highest-spend Medicare drugs: **10 drugs in 2026**, expanding to **15 in 2027** and **20 per year** thereafter. First-cohort MFPs took effect January 2026 — published average **60–80% reduction from list price**.
- 2 **Inflation rebates** — manufacturers rebate CMS if price increases exceed CPI.
- 3 **Part D out-of-pocket cap** — beneficiary OOP capped at **USD 2,000 per year from 2025**.

IRA exposure is highest in cardiovascular, diabetes and oncology products with significant Medicare mix.

WHO ACTUALLY PLACES THE PRODUCT

ACTOR	ROLE ON THE PUBLISHED PAGE
PBMs	UnitedHealth/Optum, CVS/Caremark, Express Scripts/Cigna — formulary and rebate negotiations for most commercial and Part D plans
CMS	Medicare Part D coverage; Part B ASP+6%; IRA MFPs; NCD for selected products
GPOs	Premier, Vizient, HealthTrust — hospital supply-chain pricing
ICER	Influences payer decisions; no statutory authority

FDA approval does not guarantee commercial access. Model PBM, CMS and GPO clocks as separate workstreams — not a single national listing date.

IRA, PBM and access FAQ on /usa-healthcare-market-report (published 27 May 2026).

08 Hospitals and named centres

Mayo Clinic Rochester — 1,265 beds; #1 US News & World Report 2024; multi-specialty.

Cleveland Clinic — 1,400 beds; cardiology #1 in USA (US News); Heart & Vascular Institute.

Johns Hopkins Hospital Baltimore — 1,090 beds; oncology, neurology, transplant; Sidney Kimmel Comprehensive Cancer Center.

Massachusetts General Hospital — 1,000 beds; #3 US News; oncology, neurology, transplant.

UCSF Health — 800 beds; oncology, neurology, transplant — West Coast reference.

Memorial Sloan Kettering Cancer Center — 514 beds; #1 US oncology centre; precision oncology, CAR-T.

CAPACITY PUBLISHED ON-PAGE

~6,120 hospitals; ~920,000 beds (2.7 per 1,000). These six centres are the published account map, not an exhaustive IDN census.

HOW BIONIXUS USES THE US PAGE

The live report is written for US-headquartered companies that need GCC/MENA granularity (SFDA, MOHAP, NUPCO, physician panels). This sample stays on US facts. Commissioned work adds PBM-specific net-price, IRA cohort exposure and hospital-level consumption — not reproduced here.

Hospital list and bed counts: BioNixus USA market intelligence on /usa-healthcare-market-report.

09 Methodology and sources

Figures are taken from BioNexus published website intelligence on **/usa-healthcare-market-report** (datePublished / dateModified **27 May 2026**) and the USA market-intelligence KPI panel rendered on that page. This sample edition: **September 2026**.

RECONCILIATION RULES

Headline chips follow the live hero: healthcare **~\$4.5T**, pharma **~\$615B**, devices **~\$180B**. FAQ bands (USD 4.4–4.6T; 590–640B) sit behind those chips. The KPI panel's CMS-style THE (USD 5.0–5.2T) and pharma (USD 615–650B) / devices (USD 175–200B) are shown as a second series — they are not averaged into a new number.

CITED SECONDARY SOURCES (AS NAMED ON THE PAGE)

US Census Bureau — population. IMF 2025 — GDP per capita. CMS National Health Expenditure Accounts — per-capita spend on the KPI panel. AHA 2024 — hospital census. ACS 2024, AHA 2024, CDC NHANES 2023 — epidemiology. MDMA/AdvaMed — device KPI note. IRA mechanics — page FAQ.

PRIMARY CONSTRUCTION (FULL PROGRAMMES — NOT THIS PDF)

PBM formulary audits, hospital consumption analogues, GPO bid tracking, IRA cohort modelling, bilingual MENA fieldwork for US companies entering GCC. This sample does not reproduce those microdata tables.

Gated excerpt only. Therapy SKU shares, rebate guarantees and hospital-level consumption remain in commissioned BioNexus work. Nothing herein is legal, medical, pricing or investment advice.

bionixus.com/usa-healthcare-market-report

NEXT STEP

Primary research, not a spreadsheet export.

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admin@bionixus.com · +20 120 688 2323 (Cairo) · bionixus.com/contact